TOBACCO CESSATION
CLINICAL GUIDELINES
for
THE GAMBIA

Ministry of Health and Social Welfare, The Gambia

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FOREWORD

In the view of the complexities surrounding tobacco use as well as the huge detrimental effects it has on socio-economic development, concerted action is required from across the different sectors in Government as well as the private sector, institutions, individuals, communities and organisations including the United Nations System to tackle them.

In The Gambia, tobacco use constitutes the most significant risk factor for Non Communicable Diseases (NCDs). Directly or indirectly, tobacco-induced cancer, disability, hean diseases, chronic respiratory diseases, diabetes, to name but a few, and their complications place an unnecessary high burden on the meager resources for the health sector.

Despite the complications enumerated above, the prevalence of smoking in The Gambia still remains high, particularly among males and adolescents, and the healthcare providers are not well trained and equipped to manage this problem effectively. Moreover, most healthcare providers don't seem to understand that treating tobacco dependence is more cost-effective than treating tobacco-related diseases.

Therefore, reducing smoking prevalence is key to improving people's health and reducing health inequalities in The Gambia and the development of smoking cessation clinical guidelines is part of an integrated, national NCO programme of action that will contribute to reducing the smoking prevalence. Much has already been achieved in helping smokers to quit, but much more still needs to be done.

Hence, the Ministry of Health and Social Welfare deems it prudent to develop clinical guidelines for tobacco cessation for both public and private health care providers to better equip them to tackle this problem effectively.

Since prevention and control of tobacco use is beyond the purview of a single entity, the Ministry of Health recognizes the need for health care providers and other professionals to participate in smoking cessation programmes. The guidelines provide the basis for cessation services over the next three years. It also provides a range of evidence-based strategies that can improve the implementation of effective smoking cessation interventions in the health care setting in addition to providing guidance on smoking cessation services and other relevant interventions.It is hoped that they will assist professionals in providing better and affordable services to those who are in need of help to quit smoking.

Providing such a systematic approach to smoking cessation is associated with higher levels of success in other countries. It is also envisioned that The Gambia will equally benefit should they be implemented in the country. Therefore, on behalf of the Ministry of Health and Social Welfare, I hereby support the full adoption of this three-year Tobacco Cessation Clinical Guidelines for The Gambia and call on all partners to support in its implementation.

Hon Omar Sey
Minister of Health and Social Welfare

MINISTRY OF HEALTH & SOCIAL WELFARE
PERMANENT SECRETARY

Acknowledgements and authors

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Finally, we would like to take this opportunity to express our sincere thanks to the Honorable Minister of Health and Social Welfare, the Hon. Omar sey, for honouring us by giving his fullest support to the implementation of this tobacco cessation clinical guidelines.

These guidelines were authored by Omar BADJIE, Yvonne OLANDO and Dr Ahmed E. Ogwell OUMA.

Executive summary

The Gambia has made progress in tobacco control by implementing various initiatives including the Prohibition of Smoking (Public Places) Act 1998 and the Ban on Tobacco Advertisement Act 2003; signing and ratification of the WHO Framework Convention on Tobacco Control (WHO FCTC) in June 2007; surveillance of tobacco use through the Global Youth Tobacco Survey (2008); awareness creation such as marking of World No-Tobacco Day every year; formulation of policies and laws to protect members of the public from tobacco use; increasing the prices of tobacco products through taxation; and establishing a tobacco cessation program in the Gambia.

The Gambian government in collaboration with World Health Organization (WHO) and other stakeholders launched a tobacco control campaign to eradicate tobacco use and its related diseases. The initiative calls for mass media campaigns with the theme ***"Smoke free Gambia"***. Treatment of tobacco dependence and cessation is one such aspect of the campaign.

Accumulated experience shows that interventions designed to help dependent tobacco users are most effective when they are embedded within a comprehensive tobacco control strategy employing a broad range of policies and activities. Although The Gambia has made significant achievements in tobacco control, the gap in tobacco dependence treatment and cessation needs to be urgently addressed. The national clinical guidelines are going to support the closing of this gap.

These clinical guidelines are a practical and concise resource that is available for clinicians, health managers and other health care workers. The guidelines provide an understanding of the various types of interventions including brief intervention (The 5As and 5Rs) and pharmacological interventions. It explains the various strategies used to address tobacco dependence depending on the levels of addiction to tobacco.

These guidelines also address the needs of special groups including pregnant and lactating women, young users, those with other addictions as well (e.g. alcohol or hard drugs), and passive smokers. Strategies and methods on how to avoid relapse are also provided. These guidelines are part of a comprehensive tobacco control programme in The Gambia and support the progress made through other interventions like increase in tobacco taxation, banning smoking in public places and banning the advertising of tobacco products.

Introduction

Tobacco kills about half of the people who use it, and is the foremost preventable cause of death and disease in the world today (1). It kills nearly 6 million people globally, and about 0.6 million of these can be attributed to exposure to second-hand smoke. (SHS) (2). A report released in the year 2000 by the US Surgeon General emphasized that tobacco cessation is considerably more cost-effective than most other health care interventions (3). This implies that, with time, other parts of the health care system will benefit from resources saved by the implementation of tobacco cessation.

Smoking in particular is estimated to cause about 71% of lung cancer cases, 42% of chronic respiratory diseases and nearly 10% of cardiovascular diseases and stroke. It is responsible for 12% of male deaths and 6% of female deaths in the world. Smokeless tobacco, which is consumed in un-burnt form through chewing and sniffing, also contains several carcinogenic compounds and has been associated with oral cancer, hypertension, heart disease and other conditions (4). SHS is harmful and hazardous to the health of the general public and particularly dangerous to children. It increases the risk of serious respiratory problems in children, such as greater number and severity of asthma attacks and lower respiratory tract infections, and increases the risk of middle ear infections. Inhaling second-hand smoke also causes lung cancer and coronary heart disease in non-smoking adults. (5, 6)

The main type of tobacco product consumed in The Gambia is cigarettes. Other smoked products include cigars, pipes and *manis,* (a locally rolled cigarette). The *manis* are more popular among the youth since they are cheap and easily available locally. Prevalence studies on tobacco use in the Gambia conducted by WHO include the STEPWISE Survey of 2010 (7), reported the prevalence of daily adult cigarette smokers at 14.5% with 29.4% daily male smokers and 0.7% daily female smokers. The Global Youth Tobacco Survey of 2008 (8) reported that 24.5% of young people aged 13–15 years use tobacco, with insignificant difference between boys and girls (28.6% for Boys and 20.3% for Girls). These rates suggest that there is urgent need to make tobacco cessation services available so that it can contribute to the reduction of tobacco use in The Gambia.

Treatment of tobacco dependence and cessation is an essential part of a tobacco control programme. These guidelines will therefore provide a framework under which tobacco cessation will be offered in The Gambia. They are in line with the provisions of the WHO FCTC which aim to protect the current and future generations from the consequences of tobacco consumption and exposure to tobacco smoke. Article 14 of the WHO FCTC states that "*each scientific Party shall develop and disseminate appropriate, comprehensive and integrated guidelines based on evidence and best practices, taking into account national circumstances and priorities, and shall of take effective measures to promote cessation* is *committed to meet the tobacco use and adequate treatment for tobacco dependence*". The

Gambia is a Party to the WHO FCTC and obligations to the Treaty by implementing the provisions of the Convention.

The Prohibition of Smoking in Public Places Act was passed in the National Assembly of The Gambia on 25th July 1998 and assented into law on 23rd September 1998. It outlines key tobacco control measures including a total ban on smoking in all public places. Also being implemented is the Tobacco Advertisement Act 2003 which bans all forms of advertising of tobacco products, and increasing the prices of tobacco products through taxation.

The Gambia therefore introduces these clinical guidelines as part of the efforts to promote tobacco cessation; to reduce the public health burden of tobacco use; to prevent people from starting to use tobacco and to help users to quit.

Purpose of the guidelines

The purpose of these Tobacco cessation Clinical Guidelines is to make evidence-based treatment for tobacco use and dependence accessible to all health care professionals, so as to provide standardized and appropriate treatment for all tobacco users.

General objectives

To promote tobacco cessation by implementing evidence based strategies by health care professionals at all levels.

Specific objectives

1) To provide a standardized approach to tobacco dependence treatment and cessation in The Gambia;
2) To provide tools for effective tobacco cessation intervention by healthcare professionals;
3) To promote tobacco cessation among tobacco users;
4) To lower the prevalence of tobacco users in the Gambia

Target audience and role of health professionals

These clinical guidelines are intended for use by clinicians, policy makers and healthcare professionals at all levels of care. They are intended to increase the knowledge, skills and confidence of primary care service providers so that they can design and implement effective cessation services. This can include improving the integrated delivery of brief tobacco interventions in primary care settings and involving communities to support the provision of such interventions. Studies have shown that people who use tobacco are more likely to quit if their physicians counsel them using evidence-based guidelines on the treatment of tobacco use and dependence. Unfortunately, physicians identify only about half of current smokers, advise fewer than half of them, and assist an even smaller proportion (9).

Tobacco dependence

WHO's International classification of diseases (ICD-10) (10) and the American Psychiatric Association's Diagnostic and statistical manual, fifth edition (DSM-V) (APA) (11) recognizes tobacco as being an addictive drug, which leads to dependence and has a chronic relapsing tendency. The essential feature of tobacco dependence is a cluster of cognitive,[1] behavioral and physiological symptoms. The DSM-V criteria for tobacco dependence includes, a pattern of tobacco use, that is manifested by three (or more) of the following, occurring at any time in the same 12 month- period:

1. Tolerance – the need for greatly increased amounts of tobacco to achieve the desired effect or markedly diminished effect with continued use of the same amount of tobacco.
2. Withdrawal –unpleasant feelings as a result of tobacco cessation or reduction in amount normally taken. E.g. headaches, irritability, uncontrollable coughing etc.
3. There is a persistent desire or unsuccessful efforts to cut down or control tobacco use.
4. A great deal of time is spent in activities necessary to obtain the tobacco, use it or recover from its effects.
5. The tobacco use is continued despite knowledge of having a persistent recurrent physical or psychological problem that is likely to have been caused or exacerbated by the substance.

The DSM-V criterion for tobacco withdrawal includes any four of the following when caused by the lack of nicotine: - depressed mood, insomnia (lack of sleep), irritability, frustration, anger, anxiety, cravings, difficulty in concentration, restlessness, decreased heart rate and increased appetite or weight gain.

[1] Cognitive meaning-relating to the mental processes of perception, memory, judgment, and reasoning

The symptoms above should not be due to any other general medical condition or disorder. Tobacco withdrawal symptoms typically resolve over 10 to 14 days but can last up to 4 weeks and association that causes the person to think about smoking (triggers) can make it persist for longer.

The Fagerstrom Test for Nicotine Dependence (12) is a standard instrument for assessing the intensity of the physical addiction to nicotine. This test helps healthcare professionals document the indications for prescribing medication for nicotine withdrawal. The higher the Fagerstrom score, the more intense the patient's physical dependence on nicotine. Higher scores indicate that to treat withdrawal symptoms, in most cases nicotine replacement therapy will be an important factor in the patient's plan of care. (Appendix 1)

Benefits of quitting

When a person stops using tobacco, the body begins to heal and this reduces the risks for cardiovascular disease and cancers including; cancers of the esophagus, larynx, kidney, pancreas, and cervix. (13). Within the first 24 hours after quitting tobacco use, blood pressure and pulse rate return to normal, nicotine and carbon monoxide levels in blood are reduced by half; oxygen levels return to normal, and carbon monoxide is eliminated from the body. Lungs also start to clear out mucus and other smoking debris. Within 2 days, there is no nicotine left in the body, ability to taste and smell is greatly improved and by the third day breathing becomes easier. Bronchial tubes begin to relax and general energy levels increase. Other long term benefits include improved fertility, whiter teeth, healthier family members (reduced SHS), increased energy and better sex. (14)

Barriers to quitting

Concerns or barriers to quitting are important for all tobacco users. An informed discussion can be very helpful to assist tobacco users to overcome these, by providing information and correcting misconceptions in advance.

The common barriers to quitting and their possible counteractions include:

Perceived barrier to quitting	**Possible counteracting activities**
Withdrawals and cravings (Headaches, nosebleeds etc.)	• Exercises and distractions • Nicotine Replacement Therapy (NRTs) in severe cases
Stress	Learning relaxations skills such as: • imagery, • listening to music, • deep breathing exercises and simple day to day

	exercises
Fear of failure (relapse)	Tobacco users should be helped to explore some challenges that they faced in the previous quitting attempts. • Coping strategies for high-risk situations should be discussed. • Pharmacotherapies, counseling and support groups should also be suggested as alternatives in the quitting process
Weight gain	• A balanced low fat diet and avoidance of high-fat foods, • Drinking water or low-calorie drinks as a substitute to snacking, • Regular exercises, • identifying eating triggers can be used to cope with this.
Peer pressure	• Patients should be encouraged to spend more time with non-tobacco users, or spend more time in places where tobacco use is forbidden.

Interventions for tobacco dependence

Approximately 70% of smokers' worldwide report that they want to quit; however, only one-third of them try to stop smoking each year, and less than 5–6% are successful in the long term when they attempt to quit on their own. (15) Tobacco users trying to quit need supportive therapy from trained professionals.

Tobacco cessation interventions are less costly than other routine medical interventions such as treatment of high blood pressure, cancer and other diseases which are associated with tobacco use. Health care professionals should provide counseling interventions for patients who meet the criteria for tobacco dependence. Multi-contact counseling is more effective than single-contact counseling interventions, but healthcare professionals should tailor counseling intensity to address individual patient needs. Intensity is determined by the duration of each contact and whether any follow-up occurs. Tobacco cessation programs often combine behavioral support (such as psychological interventions, telephone support, and self-help) with drug treatment where necessary. Before deciding on which intervention to use, it is essential to document tobacco use status and conduct screening.

Categories of tobacco cessation interventions:

There are three main categories of intervention: brief advice by a healthcare professional, face-to-face behavioral support and pharmacotherapy. (16, 17, 18).

Behavioral support

a) Cognitive and Behavioral interventions (CBT)

CBT is a form of therapy whose strategies aim at changing thought processes and beliefs. CBT for quitting tobacco use focuses on changing thinking patterns. If one changes the way they feel, about tobacco use, a change in behavior should follow. The healthcare professional helps the patient deal with negative feelings such as hopelessness and self-criticism which the patient may harbor especially after a relapse. It's essential that goals set by the patient be realistic to avoid failure.

CBT techniques for quitting may include;

1. Education about the quit process
2. Individualized problem-solving strategies e.g. changing thinking patterns, Aversion therapy (having pictures of a young lady with wrinkles holding a cigarette)
3. Identifying social or environmental cues that trigger the urge for a cigarette
4. Identifying motivational cues (health benefits, money saved from quitting)
5. Determining the level of social support the patient will have while attempting to quit tobacco use.

CBT after relapse:

This is a cognitive exercise and the following questions would be helpful:

- How were you successful before? For example, what were you thinking and doing when you were not thinking about smoking?
- What worked that you would like to use in your next quit attempt?
- What triggered you to go back to tobacco use?

b) Behavioral strategies

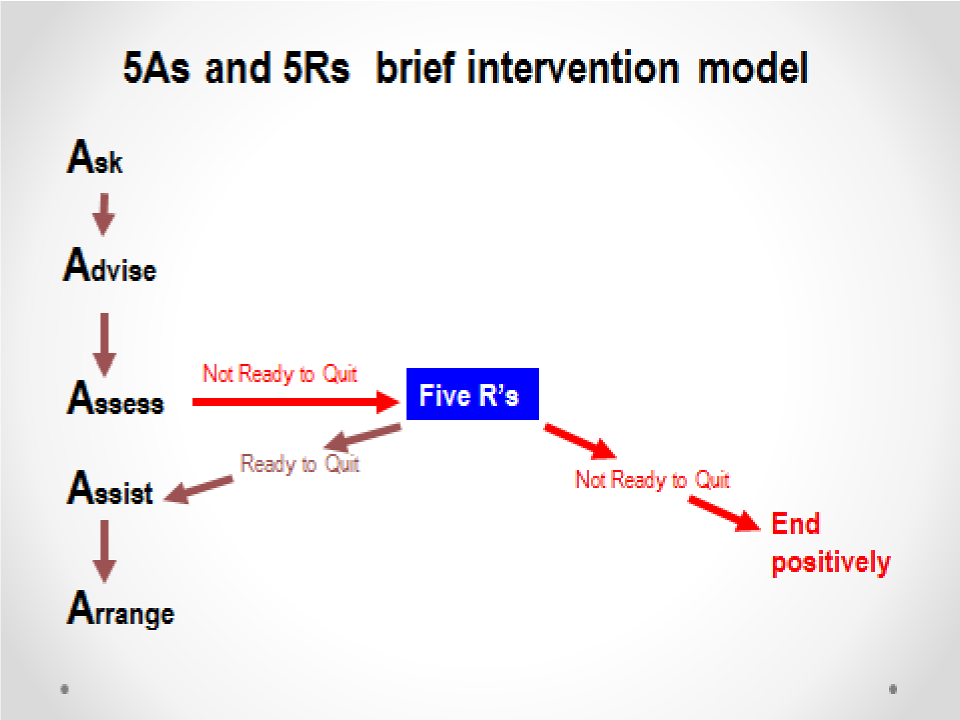
A number of behavioral strategies can support a patient to cope with the triggers and high-risk situations for tobacco use. Ideally, the patient should suggest their own alternatives and substitute activities including taking part in some exercises.

Behavioral strategy	Action
Individual behavioral counseling	• Scheduled face-to-face appointments with a trained tobacco cessation healthcare provider • Use motivational interviewing to elicit change talk in clients • Continue with weekly sessions for a period of at least 4 weeks before the planned quit date.
Group behavior therapy	• Group participants provide peer

	support to each other. Older patients who have quit are encouraged to act like sponsors to new patients.
Telephone counseling/Quit lines	• Increased frequency of calls to the quit line increases the likelihood of the caller quitting. • Telephone counseling may be a good option for people with limited financial resources. It can reach large numbers without medical referral and can be incorporated as a part of smoking cessation service. • Healthcare professionals to encourage use of quit lines especially when triggers occur in patients attempting to quit.
Self-help interventions	• Includes structured programs that are used by individuals without the help of health professionals, trained counselors, or support groups. These are usually written materials provided by government health departments and organizations dealing with addiction cessation; which could be leaflets, videos/DVDs, audio recordings or Internet-based materials. Current evidence suggests there is likely to be a small effect from the use of standard self-help materials on quit rates compared with no intervention. (19) • Self-help materials to be made readily available and accessible in health care settings to encourage awareness promotion and sensitization on tobacco use effects.

c) Brief therapy (The 5As and 5Rs model)

Brief therapy is the focused application of therapeutic techniques specifically targeted to a symptom or behavior and oriented toward a limited length of treatment. The tobacco cessation research (20, 21) strongly supports the use of a comprehensive, clinic-based approach to tobacco cessation, known as the 5A's— ask, advise, assess, assist, and arrange follow-up. The following steps are recommended as the "5As" for effective intervention for tobacco cessation in current users.



Step 1: ASK	**Identify the tobacco users** • Healthcare professionals have to ask the patient about tobacco use at every clinic visit. • Two questions are important: (i). Whether the patient uses tobacco currently, (ii). Has used tobacco in the past, and, if so,(iii). Whether they are currently interested in stopping.
Step 2: ADVICE	**Give advice** • Advice on the benefits of stopping tobacco use and the health risks of continuing use of tobacco should be given in a personalized manner.
Step 3: ASSESS	**Assess motivation to quit smoking** Three questions will help healthcare professionals to recognize motivated quitters and get them started on an appropriate action plan:

<table>
<tr><td></td><td>1. Does the patient want to stop tobacco use?
2. How important is it for the patient to stop using tobacco?
3. Would the patient be prepared to stop tobacco use in the next 2 weeks?</td></tr>
<tr><td colspan="2">If during the assessment the patient is clearly not motivated to make a quit attempt, then the health care professional should introduce the 5Rs.
The 5Rs include:
Relevance - How is quitting most personally relevant to you?
Risks - What do you know about the risks of tobacco use?
Rewards - What would be the benefits of quitting?
Roadblocks - What would be difficult about quitting for you? Any challenges?
Repetition - Repeat assessment of readiness to quit – if still not ready to quit repeat intervention at a later date.</td></tr>
<tr><td>Step 4: ASSIST</td><td>Help the tobacco user in quitting
• Set a quit date, ideally in 2 weeks
• Provide practical counseling (e.g. How to manage cravings, how to say no to tobacco using friends etc)and skills training (e.g. anger management, stress management etc)
• Discuss possible nicotine withdrawal symptoms and how the patient can successfully overcome them, particularly during the critical first two weeks.
• Identify other related problems and plan how to cope with them.e.g. grief, loss and frustrating situations.
• Ask family and friends for support
• Make a personalized action plan with treatment recommendations.
• Information relating to how to stop can be reinforced with leaflets, booklets or other self-help materials.
• Total abstinence should be strongly emphasized: “not even a single puff after the quit date”.</td></tr>
<tr><td>Step 5: ARRANGE</td><td>Arrange a follow-up contact.
• Follow-up visits should occur soon after the initiation of the tobacco cessation intervention near the quit date, preferably during the first week, with a second visit within the first month.
• Telephone contact may also be helpful.
• At each follow-up, contact success should be congratulated, and problems and difficulties should be identified to help facilitate the patient’s attempt.
• Ideally, cessation should be validated in 1 month and then again in 3, 6 and 12 months.</td></tr>
</table>

Pharmacological interventions

Medications are an important and effective tool for increasing cessation success by relieving nicotine craving and withdrawal symptoms. Medical providers should encourage tobacco users who are planning a quit attempt to use one or a combination of cessation medications, except when medically contraindicated.

i. Nicotine replacement therapy

Nicotine replacement therapy is the remedial administration of nicotine to the body by means other than tobacco use, usually as part of tobacco cessation. The primary benefit of nicotine replacement therapy is that it prevents cravings in a tobacco user whilst allowing them to abstain from tobacco. The person making the quit attempt therefore receives nicotine while avoiding the other harmful chemicals found in tobacco.

Nicotine replacement therapies are found in different forms:

Forms	**How it works**	**Available dosage**
Nicotine gum	Delivers nicotine through the lining of the mouth	2mg, 4 mg
Nicotine patch	Delivers nicotine through skin	24 hour delivery systems 7mg, 14mg, 21mg 16 hour delivery systems 5mg, 10mg, 15mg
Nicotine lozenge	Delivers nicotine through the lining of the mouth	2mg, 4 mg
Nicotine nasal spray	Delivers nicotine through the lining of the nose	0.5 mg nicotine in 50 µl aqueous nicotine solution
Nicotine inhaler	Delivers nicotine to the oral mucosa, not the lung	10 mg cartridge delivers 4mg inhaled nicotine vapor

ii. Varenicline

Varenicline is orally administered. It operates in two ways. Acting as an "antagonist", it blocks nicotine's connection to receptors in the brain, making smoking less satisfying and desirable. At the same time, as an "agonist", Varenicline mimics the effects of nicotine, therefore reducing the cravings and withdrawal symptoms. It is usually started 1-2 weeks before the quit date. If nausea is experienced during this period, one should try to reduce tobacco use as much as possible. Varenicline should be taken with food and water (at least one full glass) at each dose. If lapses occur, current dose should be continued as well as efforts to quit tobacco use. It is taken for 12 weeks

- 0.5mg once daily for first 3days
- 0.5mg twice daily for the next 4days
- 1mg twice daily starting day 8 all the way to the 12 weeks

There are no significant clinical side effects noted though cases of depression, suicidal ideations and myocardial ideation infarction have been noted. (22) Varenicline is not recommended for pregnant and lactating mothers.

iii. Bupropion

Bupropion is an extended-release medication that reduces symptoms of nicotine withdrawal. It acts on chemicals in the brain that are related to nicotine craving, but it does not contain nicotine. Bupropion may be a good choice for people who prefer an alternative to nicotine replacement, or who have not succeeded on traditional nicotine replacement therapies. It is effective for both genders and has been shown to aide cessation in depressed patients. Bupropion is generally well tolerated in persons with cardiovascular disease. Treatment with bupropion begins while the user is still using tobacco, one week prior to the quit date. Recommended dosing is as follows:

- 150 mg/day, given every morning for the first 3 days
- 300 mg/day, given as 150 mg twice daily (morning and evening).

Some persons do well on 150 mg once a day and have fewer side effects. An interval of at least 8 hours between successive doses is advised. The quit attempt should occur during second week of treatment.

Smoking interventions in special groups

1. Pregnant and lactating women

Women who use tobacco have more miscarriages and stillbirths, and their babies experience a higher rate of sudden infant death syndrome than the general public. Infants born to tobacco users are also at a higher risk of brain damage, low birth weight, and respiratory disorders. Children exposed to secondhand smoke are more likely to suffer from health problems, including pneumonia, bronchitis, ear infections, and asthma.

Health care professionals who look after pregnant women are in a better position to offer them tobacco cessation interventions. This is because, women are often offered support by health care professionals in making positive changes to nutrition, breastfeeding, exercise and other important issues related to pregnancy and childcare and in the same way they could also be offered support in quitting tobacco use. Health care professionals need to be sensitive to the fact that some pregnant women may be hesitant to discuss their tobacco use status for fears of stigmatization, or having feelings of guilt. These primary health care givers can also offer or refer the women to more intensive and ongoing counseling. The interventions should be tailored to the specific needs of the woman and address the effects of tobacco use during pregnancy. NRT can be considered for use in pregnancy and during breastfeeding if one is unable to quit tobacco use.

2. Younger tobacco users

Studies indicate that most teenagers and young adult smokers want to quit and try to do so, but few succeed. (23)The Centers for Disease Control and Prevention (CDC) recommends that one of the major goals of any tobacco control program should be to promote quitting among both young people and adults. CDC also recommends that comprehensive school health programs should include efforts to help students and school staff members quit. (24)

Interventions for the youth:

i. Health care professionals should screen pediatric and adolescent patients and their parents for tobacco use and provide a strong message regarding the importance of totally abstaining from tobacco use.
ii. Counseling and behavioral interventions shown to be effective with adults should be considered for use with children and adolescents. The content of these interventions should be modified to be appropriate for young people.
iii. Health care professionals in a pediatric setting should offer tobacco use cessation advice and interventions to parents to limit children's exposure to secondhand smoke

Other interventions include:

Mass media campaigns for tobacco control are effective in changing knowledge, attitudes and behavior among youth about tobacco use. Cognitive behavioral interventions which include changing the thoughts and beliefs of the youth around tobacco use, individual and group behavioral counseling interventions and quit lines can also be used.

3. Patients with mental disorders

Individuals with serious mental illness are dying 25 years prematurely, and the major causes of death are tobacco-related cancer, heart disease, and lung disease. (25 , 26) Also, people with mental health problems, have increased levels of nicotine dependency and are therefore at even greater risk of tobacco-related harm. (27)

Interventions in this group can be best offered by mental health professionals who should first determine if patients use tobacco. If they do use tobacco, then offer to help quit attempts by providing proven cessation interventions. Mental health professionals should be especially aware of the behavior changes that may occur with withdrawing from nicotine, and should make sure that their patients are aware of them. Medicines used to treat mental illness may need to be monitored and adjusted for those who are trying to quit tobacco use.

4. Patients with other Addictions

Drug and alcohol abusers have an extremely high rate of nicotine dependence. Concurrent treatment for tobacco and other substances is effective, and combining treatments is a useful and successful way to treat concurrent addictions. (28, 29)

5. Smoking intervention for hospitalized tobacco users

If patients are admitted due to tobacco use-related conditions such as chronic obstructive pulmonary disease, a stroke, ischemic or heart disease; cessation information and where possible treatment can be received, is important for them as stopping tobacco use will significantly reduce their risk of repeated admissions and premature death. (30) Withdrawal from nicotine needs to be recognized and treated appropriately for hospitalized patients. It is often the ward nurses who are relied on to recognize the symptoms (increased appetite, insomnia, coughing, irritability, and dizziness) and make the appropriate referral. The main treatment methods in the acute treatment setting are nicotine replacement therapy, Bupropion and Varenicline and motivational interviewing among other modes of intervention. It is important that any hospital-based service is integrated with the community it serves, because many patients will move between primary and secondary care. Knowledge of the referral pathways to each service is invaluable.

6. Second hand smoke

There are more than 4000 chemicals in tobacco smoke, of which at least 250 are known to be harmful and more than 50 are known to cause cancer. Exposure to secondhand smoke causes premature death and disease in children and adults who do not smoke.

There is clear evidence of the harms of exposure to environmental tobacco smoke in pregnancy, to children (higher rates of respiratory and middle ear infections, meningococcal infections and asthma) and adults (increased risk of lung cancer and coronary heart disease). The evidence for the health effects of passive smoking has been summarized by a number of health authorities including the National Health and Medical Research Council. (31)

Relapse prevention

Relapse prevention strategies aim to assist former tobacco users to avoid or cope with high-risk tobacco use situations. Such strategies also aim to prevent a lapse from occurring or if it occurs from becoming a full relapse to tobacco use. Suggested strategies include: Identifying high-risk tobacco use situations and important triggers, planning coping strategies in advance, considering lifestyle changes that may reduce the number of high-risk situations encountered e.g. Stress management, reduction in alcohol consumption, encouraging patients to have a plan for how to deal with a slip to prevent it becoming a full relapse

Establishing tobacco cessation services

I. Education institutions

Strategies of intervention in educational institutions require a multi-disciplinary approach from teachers, support staff, school nurses, counselors, young people, government departments, and youth support groups among others.

- Educational institutions should have tobacco free policies which will apply to everyone who uses the premises for whatever reason and at any time. This policy should be widely publicized to create awareness to everyone using the premises. A 'TOBACCO FREE' sign should be posted in all places.
- Information about the health and social effects of tobacco use should be integrated into the curriculum.
- Interventions that aim to prevent the uptake of tobacco use as part of drugs education and activities related to healthier learning environments should be developed. Such interventions should be entertaining, factual and interactive. They should also aim to develop decision making skill, be tailored to age and ability, include strategies for enhancing self- esteem and resisting the pressure to use tobacco from the peers, media, family and tobacco industry. It is also essential that learning institutions involve parents and local partners involved in tobacco use prevention and cessation activities.

Research suggests that school-based prevention programs drawing on multiple strategies can be effective in reducing tobacco use, and are more effective than single-strategy approaches. (32)

II. *Health facilities*

Healthcare professionals should be role models to the patients and advice patients on cessation. Healthcare facilities should be made tobacco free zones and include tobacco cessation treatments as part of an overall health treatment strategy.

- There should be posters with 'NO TOBACCO USE' signs posted in different places in the facilities in full view of everyone accessing the facilities.
- All patients to be screened for tobacco use
- All patients to be offered brief cessation therapy
- Self-help materials to be readily available and accessible to everything accessing the facilities

III. *Workplace settings*

- Workplace settings should be declared as smoke free environments.
- Those who are tobacco dependent should be able to get access to quit aids (tobacco cessation counseling services and pharmacotherapy)
- Employee assistance programs, public health intervention and awareness should be rolled out to sensitize employees on the effects of tobacco use, policy guidelines and interventions available.

IV. *Sporting facilities*

The guidelines on the WHO FCTC framework (Article 8) *require 100% smoke-free environments in all indoor public places, indoor workplaces, and other public places. The Article 8 Guidelines urge Parties to also create 100% smoke free environments in outdoor or quasi-outdoor spaces*. (33)

- Sporting arenas be declared smoke free zones where no persons should engage in selling of any tobacco products or use of tobacco products. This to a great extent is put in place to ensure that the nonsmokers are protected from the secondary smoke exposure.

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